



MEDLEX

Where Medicine Meets Justice

FOR INSTITUTIONS · 2026-27

We build the capacity. Your institution keeps the *work.*

Training, policy and advisory for courts, prosecution services, ministries, health authorities and legal bodies — strengthening how medico-legal work is done inside the institution.

TRAINING

POLICY & GOVERNANCE

ADVISORY

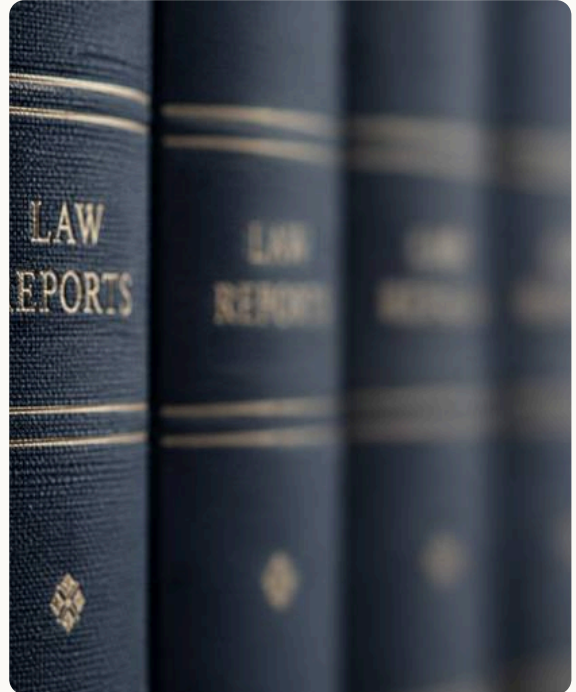
The strongest medico-legal institutions do not outsource their judgement.

Courts, prosecution services and health authorities rely on psychiatric evidence every day. Most of it is written by someone outside the institution — but it is the institution's own people who must request it, understand it and decide what to do with it.

That is where problems usually begin. A report is requested without a clear question, so it answers the wrong one. An opinion is accepted or rejected without anyone being able to explain why. Policies are written but not followed. And every difficult case depends on finding one outside expert.

MedLex strengthens the institution itself. We train your staff, improve your policies and procedures, and advise your teams on difficult cases — so the knowledge and skill remain with you after our work ends.

We do not handle cases ourselves: we never assess, write reports or give evidence. Page 4 explains this boundary and why it matters.



Who we work with

Courts and judicial bodies

Judges and court staff who must weigh psychiatric evidence.

Public prosecution

Prosecutors who instruct experts, test reports and interview vulnerable people.

Ministries and government authorities

Health and justice ministries setting standards and frameworks.

Hospitals and health authorities

Services whose clinicians write reports and answer to investigations.

Law firms and legal departments

Lawyers who commission, read and challenge psychiatric opinion.

Universities, academies and international organisations

Partners building professional capacity at scale.

Three services. One way of working.

All of it works the same way: your people, your cases, your name on the output. We work behind the scenes to make what you already do stronger and more defensible.

1

Training

Programmes designed for your institution and delivered to your teams, in Arabic or English.

- Writing psychiatric evidence that answers the legal question
- Reading and testing psychiatric evidence — for judges, prosecutors and counsel
- Risk formulation, capacity and criminal responsibility
- Interviewing vulnerable witnesses and children
- Mental health legislation in practice
- Clinical leadership, communication and internal investigations

2

Policy and governance

The frameworks that decide whether medico-legal work in your institution holds up: how opinions are commissioned, written, reviewed and recorded.

- Standards for commissioning and quality-checking psychiatric reports
- Record-keeping, confidentiality and disclosure policy
- Detention, restraint and incident-review governance
- Internal peer review and supervision structures
- Competency frameworks for medico-legal roles

3

Advisory and case discussion

A consultant in the room with your team when a case is difficult — helping them reason it through, and find the weak point before someone else does.

- Confidential case discussion with your clinicians or lawyers
- Structured reasoning and formulation support
- Review of your own draft frameworks and methods
- Standing advisory access for a defined period
- Guidance on when specialist independent input is needed

Services can be taken separately or combined — for example, a governance review followed by training on the new standard.

What we do not do

MedLex is an education and consultancy company, not a clinical or expert-witness practice. That line is deliberate, and we hold it in every engagement.

It means we are never a party to your proceedings, never in competition with your own experts, and never in a position where advising you today conflicts with an opinion we gave yesterday.

What we build stays with your institution: your people, your methods, your name on the work.

Where a case genuinely needs an independent psychiatric opinion, we say so — and help you specify what to commission, and how to test what you receive.

We never:

- ✗ Assess a patient, defendant or claimant directly
- ✗ Write or co-sign psychiatric or medico-legal reports
- ✗ Give evidence, orally or in writing, in any proceedings
- ✗ Appear before a court, tribunal or prosecutor
- ✗ Attach our name to any opinion used in a case

What we do instead: we help your own clinicians plan, carry out and review their assessments — discussing the case with them behind the scenes, as advisers, never as assessors.

This is written into every contract.

Why institutions value the boundary

No conflict

We can advise any part of your institution, on any case, without ever becoming a witness in it.

No dependency

Each engagement leaves your own people more capable, not more reliant on us.

Clear accountability

Decisions and opinions remain with the professionals who hold responsibility for them.

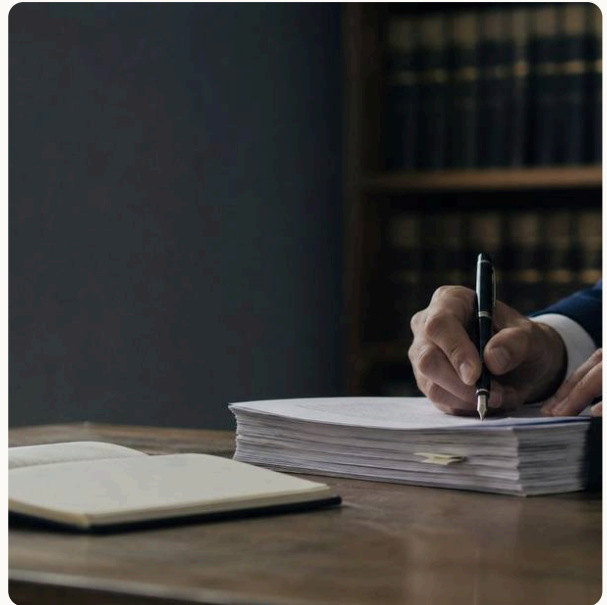
DELIVERED

TRAINING · COMPLETED

Public prosecutors, Egypt — interviewing children

Dr Ahmed Abouelghit (founder, MedLex) designed and delivered a structured training programme for public prosecutors on interviewing children, in partnership with UNICEF Egypt.

- Curriculum built from the forensic evidence base and adapted to Egyptian practice
- Delivered entirely in Arabic, to prosecutors in their own working context
- Run across multiple cohorts, with materials left with the institution
- Training only — no case involvement of any kind



HOW WE TEACH

Built for your people, not adapted from a syllabus

Designed around your practice

Cases, documents and decisions drawn from the settings your teams actually work in.

Arabic or English, written natively

Each language version written in its own right — never a translation of the other.

Practice, not lectures

Simulated case material, structured exercises and work produced during the programme.

Materials that stay

Checklists, templates and reference material left with the institution for future cohorts.

Taught with English law as the reference framework, and adapted to the legal and health system of the institution concerned.

Three ways to work with us

Every engagement is scoped in writing before it begins: what is delivered, to whom, over what period, and what is out of scope.

01

A defined programme

A training programme designed for your institution and delivered to an agreed number of cohorts — in person, online, or both.

Typically 2–8 weeks · one session a week, or a concentrated block of days

02

A governance project

A piece of work with one defined output: a policy, a framework, a set of standards, or a review of what you already have — delivered as a document your institution owns.

Typically 4–12 weeks · one named deliverable

03

Standing advisory

Scheduled access to a consultant for case discussion and guidance over an agreed period — regular sessions with your team, on your cases, with no written opinion from us.

Renewable term · agreed session frequency

What happens after you enquire

1

We reply within two working days

Usually with two or three questions about the institution and what it needs.

2

A short call

To understand the people involved, the problem, and the constraint you are working within.

3

A written proposal

Scope, delivery, language, timeline and fees — and, explicitly, what is out of scope.

4

Contract and delivery

With MedLex for Training and Consultancy LLC. Fees are quoted in USD or EGP.

WHO LEADS THE WORK



Dr Ahmed Abouelghit

UK-trained Consultant Forensic Psychiatrist · Founder, MedLex

His practice spans forensic psychiatry, medico-legal reporting, Mental Health Tribunals, court and Parole Board work — alongside directing specialist training and leading clinical services across the United Kingdom, Egypt and Qatar.

He is formerly an examiner for the Royal College of Psychiatrists' CASC, and designed and delivered training for public prosecutors in Egypt in partnership with UNICEF Egypt.

Every institutional engagement is led by him personally, from the first call to the final session.

THE MANDATE

To establish forensic and medico-legal psychiatry as a recognised discipline in the region — and to bring its standard to the rest of a clinical career.

Method before conclusion

A defined question, documented reasoning, a defensible answer.

Legal alignment

Evidence written to answer the legal question posed.

Professional protection

Reasoning that holds when it is challenged.

Regional relevance

International standards, applied to local practice.

“The measure of an institutional engagement is what your people can do after we have gone.”

MAKE AN ENQUIRY

Tell us what your institution needs.

We reply within two working days. If the work is outside what we do, we will say so — and point you to the right place.



Scan to enquire

EMAIL

info@medlexsolutions.com

ONLINE

medlexsolutions.com/institutional

Please do not send case material, patient information or identifying details with an enquiry. A short description of the need is enough to begin.



MEDLEX

Where Medicine Meets Justice

MedLex for Training and Consultancy LLC, trading as MedLex Solutions

Tax registration 774-030-542

5 El-Fadl Street, Office F1-03-02, Abdeen, Cairo, Egypt